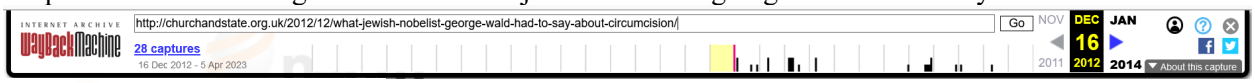
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What Jewish Nobelist George Wald had to say about circumcision

Editor's note: *Here is perhaps the single most useful paper on circumcision for those of the Jewish faith, who will consider the issue rationally, to read. The essay by Nobel Laureate George Wald (1906-1997), one of the greatest biologists of all time, and a practicing Jew, comes from Genital Autonomy: Protecting Personal Choice by George C Denniston, Frederick M Hodges and Marilyn Fayre Milos (Springer, 2010, pp. 217-239). This is what Dr Wald thought about circumcision.*



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George Wald

Abstract: At the 7th International Symposium on Circumcision, Genital Integrity, and Human Rights, held at Georgetown University in Washington, DC, Van Lewis presented a paper about the amazing work of George Wald (1906–1997), who, in 1961, won the Nobel Prize for his discovery of Vitamin A in the retina of the eye and how it works with light to produce vision. Not only was he a world-class research scientist, Time magazine declared him one of America's Ten Best Teachers. A child of immigrant New York Jewish parents, beloved Harvard biology professor, and well-known twentieth century activist for peace and justice, George Wald also worked to defend the right of all children, male and female, to genital integrity. Here, thanks to Van Lewis and George Denniston—both students of George Wald—and to his wife for giving us permission to publish this never-before-published paper. This is what Dr. Wald had to say about penile reduction surgery, euphemistically called circumcision.

George Wald (1975)

Every year in early February, after my last lecture at Harvard, I go off into the back country for a while, to put myself together. Last February it was to Mexico, to visit two remote Indian tribes in the Sierra Madre. On the way I stopped off for a lecture at Florida State University in Tallahassee. It was a big public lecture, with a discussion afterward, and a reception.

Toward the end, a young man came up, bearded, lots of hair, open shirt, jeans.[1] He introduced his wife and his mother, a stately woman, carrying the young couple's seven-month-old infant. "When are you leaving Tallahassee?" he asked. I told him on a seven o'clock flight to Atlanta next morning, to catch the Mexico City plane. "Can I drive you to the airport?" "Yes, thanks," I said thoughtlessly, "if your car will run." "We have three," he said, "and one of them is sure to run."

So at six next morning he came for me and we set out for the airport. A little way along he said, "Have you thought much about circumcision?"

"No," I said, a little surprised, "I haven't thought about it at all."

"Well, I've thought about it a lot," he said, "I've been thinking about it for years. I think it's a terrible thing to do to a male infant that's just gone through the struggle of being born, that's just left the warmth and security of the womb to come out into a cold and strange world, to greet him with the knife, with a mutilation. I've never been able to forgive my mother for having that done to me." (Note: his *mother*—a recurrent theme)

"A few years ago," he went on, "I realized that to make further progress in my thinking I'd have to go public. So I prepared some carefully lettered signs; and since it was a windy day I asked my younger brother to come along and help carry them. I told my father what we planned to do. "Well son," he said, "you know I've backed you in almost everything, but I think I'll pass this one up."

"We drove to the entrance to a big general hospital on a main road, and began to picket. One of the signs read, CIRCUMCISION IS A SEX CRIME. Another read: SEX CRIMINALS FOR HIRE? INQUIRE WITHIN.

"Almost every car that drove past would slow up to read our signs. Then something interesting developed. There was a difference in the way men and women reacted. Some of the men were with us. They would lean out, wave, and say things like, "You tell them, kid!" and "Right on!"

“But the women were furious. They shook their fists at us, and some of them stopped to curse us out. You’d be surprised at the language they used.

“After awhile the police came and took us in for disorderly conduct. We spent the afternoon in jail, posted bond, and went home.”

By that time we had reached the airport. My plane was late, so we sat down and went on talking. Up until then I had been listening, interested, a little amused, not involved. Suddenly he said something that shook me.

It seems to me,” he said, “that the foreskin is the female element in a male. It’s warm flesh enclosing the penis; a kind of male vagina.”

“My god!” I said, “That’s wonderful! Because we’ve always been told that the clitoris is the male element in a female!” And I told him about the Dogon.

The Primitive Event: An Initiation Rite

The Dogon are a West African people living in Mali south of the great bend in the Niger River. I became interested in them long ago, through their very distinctive wood sculpture. I had hopes to visit them last April; but just before setting out was warned that April is the worst month of the year in that region: temperatures near 110° and torrential rains that might make the roads impassable. So I had to give it up.

The Dogon have an extraordinary creation myth. The primal god, Amma, made the Earth from clay in the shape of a woman lying on her back. Then Amma, being lonely, wanted to copulate with her. Her vagina was an ant hill; but beside it was her clitoris, a termite mound. (These characterizations became clearer to me when I ran across a photograph of a field with termite mounds. They are not broad, rounded eminences like ant hills, but tall, slender, phallic columns.)

As Amma approached the Earth to copulate with her, the male element, the termite mound, rose against him. So first he had to destroy it.

Suddenly everything fell into place. The Dogon, like many other African peoples, not in early infancy but at or near puberty, as an initiation rite, circumcise the boys, and *excise* the girls: the clitoris is cut away, in some tribes along with the *labia minora*. Up to puberty every Dogon child is thought to be to a degree bisexual, a gynandromorph; and that is acceptable, since it has as yet no serious sexual role to fulfill. But then, in preparation for adulthood, the boys are made altogether male by removing the foreskin, their female member; and the girls are made wholly female by excising the clitoris.

One does not have to improvise this interpretation. The tribal traditions state it plainly. So, speaking of the creation of man: “each human being from the first was endowed with two souls of different sex. In the man the female soul was located in the prepuce; in the woman the male soul was in the clitoris. . . The dual soul is a danger; a man should be male, a woman female. Circumcision and excision are the remedy.”[2]

I have no doubt that this is the dominant primitive meaning of circumcision and excision: that, androgynous to a degree in infancy, children have their sex roles established unequivocally at or near puberty by removing the foreskin from boys and the clitoris from girls.

These practices are ancient and widespread. They have arisen on every continent. “The bodies of Egyptians exhumed from the earliest prehistoric cemeteries, back of 4000 B.C., have disclosed the evidence of circumcision whenever the body is sufficiently well preserved to make the observation possible. The actual performance of the operation by the Egyptian surgeon is depicted in an Egyptian tomb relief of the twenty-seventh or twenty-eighth century B.C. in the cemetery of Memphis.”[3] This great Egyptologist believed that the ancient Hebrews, led by Moses, “born in Egypt and bearing an Egyptian name” (Mose = child of, as in the Pharaonic names Ahmose, Thutmose), borrowed from the Egyptians at once the Pharaoh Ikhnaton’s monotheism, the rite of circumcision, and the ban on eating pork. Yet among the ancient Egyptians also, circumcision was a puberty rite, performed at ages 6-14.

It is curious that up to relatively modern times, circumcision never set the Jews off from most of the people about them. The custom prevailed not only among the ancient Egyptians, but the Semitic peoples among whom the Jews continued to dwell: Moabites, Edomites, Ammonites, Phoenicians. Circumcision tended much more to divide Semites from non-Semites than Jews from others. To the ancient Jews the epitome of the uncircumcised were the Philistines, a non-Semitic sea people, probably from Crete; until the ninth century B.C. they encountered also the Assyrians, Semites yet uncircumcised.

With the coming of the Prophet, circumcision became universal among Moslems, accompanied in some groups by female excision. It is practiced ritually by numerous people of central and west Africa including the Ethiopians; many Australian aborigines; Malays, Fijians and Samoans; and Indian tribes in North and South America. (I have a fine pre-Columbian stirrup-bottle from the Vicus area in Peru, the spout of which has the form of an erect, circumcised penis.)

The most usual status of circumcision among all these peoples is as an initiation rite, performed at or near puberty, often in direct preparation for mating or marriage. There is some reason to believe that it may have begun that way among the ancient Hebrews. In the Ethiopian (“Coptic”) Christian Church, though boys are circumcised in early infancy, girls are excised at or close to puberty. So far as I know, no other people circumcise as early as the Jews—on the eighth day—except for present-day Americans, who owing to the exigencies of hospital practice, are likely to have their infants circumcised on the third or fourth day.[4]

Running through all modern discussions of circumcision is the thought that it began, and still operates as an aspect of preventive medicine. Adults who need to be circumcised because of some penile disorder have always uncleanness to blame for their trouble. John Morrison, an Australian physician, observes that in Australia ritual circumcision is practiced only by those tribes that live under desert conditions, in which the combination of sand, wind and shortage of water for washing would have made circumcision frequently necessary later in life, had it not been performed in childhood. He suggests that similar environments may have prevailed wherever else in the world this custom has arisen (Medical Journal of Australia, 1967, p.125).[5]

It may well be true that millennia of painful experiences had a part in developing circumcision as a ritualized health measure. That cannot be the whole story, however, or probably even a dominant motif. For one thing it does not touch the parallel practice of female excision, which no one has tried to defend on medical grounds. Nor does it apply to a great variety of other mutilations of the external genitalia practiced by native peoples. But most important of all, such surgery performed under primitive conditions must always have presented a serious hazard. Even under relatively impeccable conditions in a modern hospital, circumcision occasionally causes complications. Done with rude tools in the bush or in the desert it must often have led to infection, maiming, at times the death of the subject. It is hard to assess what net medical advantage, if any, circumcision might offer under such conditions.

It seems to me much to the point that the ancient Jews, far from looking upon circumcision as a health measure, regarded it as a dangerous operation. Thus it was decided early that a Jewish infant whose brother had died as a result of circumcision was to be spared this ritual. In the ceremony of circumcision, the special chair said to be reserved for Elijah is left in place for 3 days, because these are days of danger for the child. Moses Maimonides, the twelfth century rabbi of Cairo and court physician to Saladin, put the matter plainly: "No one should circumcise himself or his son for any other reason than pure faith; for circumcision is not like an incision on the leg or branding on the arm, but a very difficult operation."[6] To regard the Jewish rite of circumcision as primitive prophylaxis is a modern interpolation of medical hindsight on a par with the notion that the ban on eating pork was to prevent trichinosis.

As was to be expected, the rite of circumcision has also excited great psychoanalytic interest. Sigmund Freud took it to represent the symbolic castration of sons by jealous fathers.[7] There is little anthropological evidence to support this view. It implies a primitive awareness of the male role in procreation that was generally lacking, as well as an interest in castration that so far as we know developed only in relatively sophisticated peoples and in quite other associations. Bruno Bettelheim has suggested an ingenious alternative: that circumcision may represent an attempt on the part of males symbolically to mimic characteristically female roles in reproduction including bleeding at puberty.[8]

I find it a relief to turn from such baroque interpretations to the simple reasonableness of the traditional view already expressed: that, usually in the form of a puberty ceremony, circumcision is to render boys wholly male, and excision to make girls wholly female. This is, I think, the most widespread view among the peoples themselves who have practiced these rites. I think that this is as close as we shall ever come to rationalizing them.

Also I find the concept of the innate bisexuality of the human body not only attractive but well founded anatomically and embryologically. In the human fetus the external genitalia are identical in both sexes until the end of the third month. Then they begin to differentiate. The rudiment that forms the penis with its foreskin in the male becomes the clitoris with its sheath in the female. The folds that become the labia majora in the female become the scrotal sac in males. (There is no male counterpart to the vagina.) Men keep throughout life their vestigial nipples that can be developed into breasts, though never to lactate, by treatment with estrogen. (Have any native people ever excised the male nipples at puberty?)

The human body is gynandromorphic in origin, remains so to a degree until puberty, and retains vestiges of this condition throughout life. Anatomically, male and female are variations on the same central theme. That is the reality; what concerns us here are the mutilations practiced to deform that intrinsic reality.

These are amazing in their extent and variety. All peoples everywhere have displayed an obsessive preoccupation with the external genitalia (as also with the mouth: witness moustaches, painting the mouth, lip plugs, covering the mouth or veiling the lower part of the face. Eating and reproduction are the two great primal drives). The genitalia tend to be hidden, often when nothing else is hidden. Conversely, males may flaunt them as in the 16th Century European codpieces; or the gourd sheaths with which Dani tribesmen in west New Guinea hold their members erect and greatly exaggerate their length (R. Gardner and K.G. Heider: Gardens of War, Random House, N.Y., 1968).

Some of the mutilations are cosmetic: the male members are made more attractive by scarifying them and distorting them with swellings and protuberances. In some African tribes the labia minora are purposely lengthened so as to be visible externally. Some mutilations have to do with enforcing virginity: Some of the African Arabs practice *infibulation*: the entrance to the vagina is sewn partly closed so that copulation is impossible until this obstruction is removed. One male mutilation is most remarkable: Some of the Australian tribes that practice circumcision follow it later with so-called *sub-incision*: The penis is slit below for its entire

length, laying open the urethral canal, so that thereafter the man must squat while urinating, like a woman. The member is still capable of erection and copulation; but I should think must be less effective for delivering sperm to the cervix. Could this be a primitive device to limit conception? Or a particularly striking manifestation of Bruno Bettelheim's mother-envy?

It is against this background of endlessly bizarre practices and grotesque explanations that I single out as most reasonable and meaningful the view of circumcision as a puberty rite, along with the parallel excision in girls. Before puberty a degree of gynandromorphy is tolerated in both sexes; but at puberty, when sex begins really to matter, as an initiation into adulthood and preparation for marriage and parenthood, these ceremonial mutilations turn boys into pure males, and girls wholly into females.

The Jewish Rite

The Jewish rite of circumcision is something else again, being confined to males and performed in earliest infancy. Yet vestiges remain of its possible source in a puberty rite; so for example the 8-day-old infant is hailed as "chatan"—a bridegroom.

The Biblical injunction to circumcise first appears in the weightiest possible form, sealing the covenant between God and Abraham, father of nations: "And God said to Abraham. . . . This is my covenant, which you shall keep, between me and you and your descendants after you: Every male among you shall be circumcised. . . . He that is 8 days old among you shall be circumcised . . . both he that is born in your house and he that is bought with your money. . . . So shall my covenant be in your flesh an everlasting covenant. Any uncircumcised male . . . shall be cut off from his people: he has broken my covenant." (Gen. 17:9-14).

Abraham was then ninety-nine. God had no sooner finished speaking than Abraham had himself circumcised, he and his 13 year old son Ishmael—so a pubescent boy. Also all his male slaves. Did the slaves thereby become Jews? My rabbinic friend, a deep student of such matters, says "Almost". They became, so to speak, second-class Jews. Any who were freed thereafter were accepted as full Jews. On the other hand slaves who evaded circumcision had to be sold to Gentiles.

Circumcision is one of the holiest and most universal of Jewish rites, and yet it has its limits. One might think the command to circumcise so absolute as to permit no equivocation. It is astonishing to realize that on the contrary any son of a Jewish mother is fully a Jew, circumcised or not. A Jew whose brother has died as the result of circumcision is excused from this obligation. The Bible contains some other interesting vagaries.

Belated circumcision: When Joshua led the Israelites over the Jordan to claim the Promised Land, God enjoined him among the ceremonies of investiture to "Make flint knives and circumcise the people of Israel." For though all the Jews who came out of Egypt had been circumcised, that was not true of any born during the 40 years of wandering in the wilderness. So it was done then to all of the males in the nation, some of them 40 years old, at the Hill of the Foreskins. They laid over in camp until healed before moving on (Joshua 5:2-8).

Circumcision as a military tactic: When on his wanderings Jacob with his household came to the city-state of Sechem in Cannan, his daughter Dinah went to visit with the women of the city. The prince of Sechem seized and raped her; but also fell in love with her and wanted to marry her. But Jacob's sons were outraged by the indignity done their family, and wanted revenge. The king interceded for his son, and proposed that Jacob stay at Sechem and that their people intermarry. Jacob's sons replied, not until all the Sechemite men were circumcised. They agreed, and all underwent the rite.

"On the third day, when they were sore, two of Jacob's sons, Simeon and Levi, Dinah's brothers, took their swords and came upon the city unawares, and killed all the males." Then they took everything for their own, including the women and children; and brought back Dinah. Jacob reproached them for this deed, but only because it was impolitic (Gen. 34). Yet he seems to have kept it in mind, for on his deathbed he cursed their ferocity and cruelty, and he left them alone landless among all his sons (Gen. 49:5-7).

Circumcision for battle trophies: Saul offered his daughter Michal as wife to David because she loved him, but also because Saul, jealous of David's popularity, planned by a ruse to have the Philistines rid him of a potential rival. So when David modestly demurred, pleading his insignificance and poverty, Saul sent back word that all he asked as a bride price was one hundred Philistine foreskins. That made David happy. He brought the king two hundred foreskins, and married the princess (I Samuel 18:20-27).

Was Moses circumcised? An altogether astonishing passage occurs in Exodus 4: 24-26. It will be recalled that Moses, having killed an Egyptian who had mistreated a Hebrew, fled to Sinai and there married Zipporah, daughter of Jethro, a priest of Midian. While Moses was shepherding his father-in-law's flock, God spoke to him out of the burning bush, and ordered him to return to Egypt. Moses was reluctant to do so, but God insisted; and finally Moses gave in. He gathered up his family and started back to Egypt. Now the amazing passage: "At a night encampment on the way, the Lord met him and sought to kill him. Then Zipporah took a flint and cut off her son's foreskin," and smeared the blood on Moses's genitalia (my translation says "touched his legs with it," but that is a circumlocution) saying, "You are truly a bridegroom of blood to me!" And when the Lord let him alone she added, "A bridegroom of blood because of the circumcision."

What this seems to mean is that, growing up in Pharaoh's palace as the ward of an Egyptian princess, Moses had not been circumcised, nor were his sons born in Midian. Zipporah's quick action saved his life; God was deceived by the blood upon Moses into thinking him circumcised. The Midianites were Semites, but not Jews. Zipporah was not Jewish, hence neither were her children. But she had done the right thing; and I like to think that her exultant cry meant, "Now I am fully your Jewish wife. I have circumcised our son, and have saved your life with his blood."

(There is of course a difficulty. Since circumcision was a much more ancient Egyptian than a Jewish rite, Moses should have been circumcised as an Egyptian, if not a Jew. Perhaps this was not done in order to disguise his Jewishness, to make him seem to the Egyptian nobles a more acceptable kind of foreigner. We are never told whether, after Zipporah made Moses *appear* to have been circumcised, he actually was.)

The unique feature of circumcision among the Jews, relative to all the peoples about them who seem to have practiced it as a puberty rite, is its displacement to earliest infancy. No other people have practiced ritual circumcision on infants so young; and those who approach it, the Moslems and Ethiopians, probably did so in imitation of the Jewish custom. Ironically, the Jews are now outdone in this regard by American gentiles—and some Jews—who, having to leave the hospital 3-5 days after a child is born, have non-ritual circumcisions performed as early as the second day.

Why Did the Jews Circumcise So Early in Infancy?

Moses Maimonides, the twelfth century codifier of the Talmud, gives "three good reasons:" (1) If it were postponed, the grown boy might not submit to it. (2) The young infant does not feel much pain, "because the skin is tender and the imagination weak;" and (3) the father, who is responsible for carrying out this commandment, hardly knows the infant as yet, whereas later his love for his son might tempt him to spare the boy this mutilation.[9]

This explanation, for all its practical good sense, I believe is trying to rationalize an ancient practice that must have arisen for other, deeper and more arcane reasons, more closely connected with our earlier discussion.

Male and Female in the Jewish Tradition

Let us begin with the ancient belief that the foreskin is the female element in a male. I should like first to suggest that the displacement of circumcision to the eighth day of life, as also the failure to provide any parallel rite for females, were aspects of the obdurately male orientation of Judaism. Then, having dealt with that, I will come back a way.

The Jews alone among the Mediterranean peoples worshipped one, militantly male God. This position needed constant defending, both the mono- and the androtheism. It was with good reason that God cautioned Moses, "You shall have no other gods but me . . . for I, the Lord your God am a jealous God" (Exodus 20:3, 5). Throughout the Mediterranean region the worship of the Great Mother flourished and constantly intruded: Ashtoreth (Astarte), whom Solomon was persuaded to worship in his old age (I Kings 11:5); Asherah, mother of the gods, whose worship, attended by both male and female temple prostitutes for the use of the male communicants, persisted for centuries, even invading the Temple in Jerusalem until Josiah, the great reformer, ordered the priests to remove from the Temple "all the vessels made for Baal, for Asherah, and for all the host of heaven . . . and he brought out the Asherah from the house of the Lord… And he broke down the houses of the male cult prostitutes which were in the house of the Lord . . ." (II Kings 23:4-7). Not only was the Jewish God male; all about were powerful goddess cults. As Freud put it, Judaism is a Father religion, just as Christianity is a Son religion.[10] The only relief from this exclusively male Judeo-Christian theology is in the Roman Catholic cult of Mary.

The preoccupation with maleness extends to humankind. Not only was the first human being a male; as the feminist Mary Daly remarks with some bitterness, he preempted the first childbirth under sedation in giving birth to Eve.[11] All this ancestral couple's early children were males. Females were an afterthought. After Seth was born when Adam was 130 years old, we are told that Adam lived another 800 years "and had other sons and daughters" (Genesis 5:3-4).

The literal-minded wonder where Adam's firstborn Cain and his sons in turn found their wives. Where indeed? That was not a pressing problem to the ancient Jews. Once the men were there, women would turn up as needed.

I once was told the story of a revivalist preacher who in the course of a sermon used the phrase, "There will be wailing and gnashing of teeth!" "How about me?" asked an old woman sitting up front, "I ain't got no teeth!" "Teeth?" said the preacher, "Teeth will be provided!"

That's how it was with women in Genesis—they were *provided*!

Could it have been this obsession with maleness that persuaded the ancient Hebrews to make their sons wholly male from earliest infancy by circumcising them on the eighth day? And in the same spirit to do nothing about their daughters, then or later? The Bar-mitzvah for the sons at age thirteen, and nothing for the daughters? One of the ordinances that God gave Moses on the mountain begins: "When a man sells his daughter as a slave . . ." (Exodus 21:7). Men did not sell their sons. In the daily morning prayer, now well over two 2,000 years old, Jewish men say: "Blessed art thou, Lord our God, King of the Universe, who hast not made me a woman." Women say: ". . . who hast made me according to thy will." Exultation for the men, resignation for the women. In the Judaic scheme the thing to be is male, wholly male, right from the start; and one way to achieve that is by early circumcision.

This seems to be a plausible hypothesis; it is not intended to be more, nor can it be. But in defending it I have gone too far. I want now to draw back from it somewhat, for the reality is both more complicated and more interesting. For what I have called the obsessive male-orientation of the Mosaic tradition concealed, and perhaps for that very reason tried to overwhelm, a fundamental ambiguity, a taint of the female, not only in Adam, but reflecting back upon God himself.

In the oldest Biblical account of human creation—said to have been written in the ninth century B.C., though presumably the oral tradition goes back much further—"the Lord God formed man from the dust of the Earth (Genesis 2:7). In Hebrew this is to derive man, *Adam*, from *Adamah*, the Earth, a feminine form. It is an idea held by innumerable peoples from time immemorial. We keep it still, in our expression Mother Earth.

Two other accounts of human creation are assigned by Biblical scholars to the later Priestly version, written in the fifth century B.C. In both of them man and woman are created together, as in the words: "And God created man in his own image; in the image of God created he him; male and female created he them" (Genesis 1:27). Genesis 5:2 begins with almost the same words and then goes on: ". . . and blessed them, and called their name Adam, in the day when they were created."

In both these statements there is the same curious switch of number, from "him" to "them". The Hebrew is that way too. How get from "him" to "them"? How have a "them" named "Adam"?

Does it mean that the first human creature was bisexual? And hence that God, since he shared the same image, is at once male and female?

The rabbis who made the Talmud found this a worrisome problem, troubling enough to dispose of early and put behind them. Some ingenuity was expended upon it. Moses Maimonides summed up in the twelfth century as the opinion of "our sages"—the usual expression for a preferred interpretation—that "Adam and Eve were at first created as one being, having their backs united. They were then separated, and one half was removed and brought before Adam as Eve."[12] My rabbinical mentor tells me that this kind of idea—both of a bisexual God and a bisexual first human being made in his image, was in the mainstream of Jewish mystical (Kabbalistic) thought until dismissed in the last century under the influence of German rationalism as sacrilegious or absurd.

So Judaism at its source is not as unequivocally male-oriented as at first appeared. There is room in the tradition for God the Parent as well as God the Father; and Eve may not have been born out of Adam, but born *with* him and sundered from him, the better to "Be fruitful, and multiply, and replenish the Earth . . ." (Genesis 1:28).

And infant circumcision? This makes it seem more an act of male assertiveness, perhaps all the more aggressive because the theological ground was a bit shaky. A wholly male priesthood may have insisted very early on masculinizing not only the godhead, but every other aspect of Judaism. It may have been part of that effort to render all males wholly male from earliest infancy by removing the foreskin as a female contaminant.

This view of the matter is somewhat reinforced when coupled with the otherwise strange prescription in Deuteronomy 23:1: "He whose testicles are crushed or whose male member is cut off shall not enter the assembly of the Lord." One can understand that such mutilations might make a Jew unfit to marry, but why should they exclude him from the rituals? Is it that circumcision was regarded as confirming and purifying his maleness, whereas these more drastic mutilations would destroy it, and hence would bar him, as women were barred, from direct participation in religious observances?

Is Circumcision a Health Measure?

I have already given reasons for questioning the origins of circumcision as primitive preventive medicine. In recent times this practice has been taken up widely as a supposed "health measure" by gentiles particularly in the English-speaking United States, Canada and Australia, less in Europe. Many Jews also, religious and otherwise, now defend this practice on grounds of health.

Within the last few years many physicians have gone over to the view that infant circumcision, having begun as religious ritual, now survives in Western societies as little more than medical ritual. In that sense it is often grouped with another ritualized operation, tonsillectomy.

A few distinctions are needed. Both these operations have a limited role in *therapy*, in treating specific disorders. What is now being questioned increasingly is their routine performance as *prophylaxis*, as aspects of preventive medicine.

Tonsillectomy, like circumcision, has an ancient history going back some 2,500 years.[13] Both operations—excepting circumcision done for religious reasons—have a curious class character. Not only are they restricted largely to developed nations, but within them mainly to the well to do. They are aspects of middle class privilege, evidences of affluence and social status, demonstrations of the special care that middle class parents lavish on their young. Not only do the parents frequently initiate these procedures; they may be performed more for them than for their children, to show that they are as solicitous as the Joneses. A statistic bears out this connection: in England circumcised boys are seven times more likely to have tonsillectomies in early childhood than uncircumcised boys.[14]

Though routine tonsillectomy is rapidly declining in this country, an American pediatrician could still say in 1969: "It is probably the commonest surgical operation performed today in Western civilization."[15] A physician in good position to know assures me that this is still true. And tonsillectomy is something one can do for daughters as well as sons! Even when the physician is neutral or negative toward these procedures, the parents may still request them. And the physicians' attitudes—as some of them readily grant—may be colored by the recognition that these rapid, relatively innocuous procedures pay rather handsomely. A circumcision takes about 10 min, and a dozen may be run off any morning. One of our best Boston hospitals at present charges $40 for the use of the room and nurse; and the physician bills his private patients $30 to $50 for the operation. It adds up.

One of the reasons frequently given for infantile circumcision is that it "will avoid trouble later". The medical statistics of such later troubles among the uncircumcised also have a strong class orientation. Not only are the sons of the poor less likely to be circumcised, but their lack of circumcision is much more likely to cause later difficulties. The conditions of their lives and the kinds of things they do and done to them are much more likely to foster uncleanliness of the male member, the only condition that childhood circumcision ameliorates. The statistics of penile pathology among uncircumcised men are overwhelmingly weighted toward workers, peasants, and soldiers. What should be blamed upon poverty and squalor is heedlessly ascribed instead to lack of circumcision. As one physician says, "Venereal disease is more prevalent in lower socioeconomic groups and these are the groups that are most likely to be uncircumcised. They are also the groups in which there is a poor standard of personal hygiene. The lower socioeconomic groups are also those with a higher incidence of tuberculosis; but one could be excused for doubting that the retention of the prepuce renders one more susceptible to tuberculosis."[16]

One of the most striking things said in support of childhood circumcision is that it practically rules out the development of cancer of the penis. That may be true in the United States; but a study of Javanese men, who are circumcised ritually, found among 78 cases of carcinoma, 7 carcinomas of the penis. Conversely, though almost all of Sweden's 3.7 million males are uncircumcised, in 1960 only 15 cases were found of cancer of the penis or scrotum. It seems clear that penile cancer is very rare in uncircumcised men with high standards of cleanliness as in Sweden; and that circumcision offers little protection where personal hygiene is not as prevalent. "If the uncircumcised man has a foreskin which he can retract and which he keeps clean, the risk of this cancer is removed."[17]

There was a recent flurry in the medical literature owing to the allegation that cancer of the cervix is more prevalent in the wives of uncircumcised men. The initial observation was that Jewish women have lower rates of cervical cancer than gentile women. However gentile women with circumcised husbands seem to develop cervical cancers as frequently as gentile women whose husbands are not circumcised.[18] In fact cancer of the cervix seems to follow the same class pattern as penile disorders of all types including penile cancer: "Factors shown to be associated with a high risk of developing cervical cancer include low socioeconomic status, early marriage, multiple marriages, extramarital relations, coitus at an early age, frequent coitus, non-use of contraceptives, syphilis and multiparity."[19] There is no solid basis for believing that circumcision in itself has anything to do with the incidence of cervical cancer.

To keep the penis properly clean in the adult demands retracting the foreskin. Many mothers are alarmed because their infants' foreskins cannot be drawn back. But in fact this is the normal condition in young infants. The foreskin can only rarely be retracted at birth, and ordinarily becomes retractable only at 2-3 years of age. In a careful English study the prepuce was found to be completely retractable in only 4% of newborn boys. In only 54% the tip of the glans (the head of the penis) could just be seen, while in 42% it was completely hidden. Even at 6 months the foreskin could be retracted in only 20% of the infants, whereas at 1, 2 and 3 years this figure rose to 50, 80, and 90%.[20] With increasing age the condition improves further. A study in Danish schoolboys, few of whom are circumcised, showed that the foreskin could not be retracted (phimosis) in 8% of 6-7 year olds, but only 1% of 16-17 year olds.[21] Clearly the way to deal with unretractable foreskins in boys is not to circumcise, but to wait.

Another common complaint is that the foreskin adheres to the glans. Again this seems to be normal in young boys. In Danish schoolboys Oster found the incidence of such "adhesions" to diminish without treatment from 63% in 6-7 year olds to 3% in 16-17 year olds. No adhesions were found among ninety-five 17 year olds. Normally the skin of glans and foreskin, which may be fused in infants, separates spontaneously during childhood, a process that may take to age 17 to complete.[22]

It is also normal for a cheesy sebaceous material called smegma to collect between foreskin and glans. This causes no trouble in young children with unretractable foreskins, since they form little smegma. Oster found smegma in only 1% of 6-7 year olds, increasing at about puberty, and rising to 8% in 16-17 year olds.

One can conclude that there is little trouble to expect in the uncircumcised that would not be prevented by simple cleanliness, by older boys and men occasionally drawing back the foreskin and washing gently. As one physician has remarked, the problem is little different from washing behind the ears, yet no one has suggested amputating the ears.

Finally, it should be understood that circumcision, like any other surgical procedure, can *cause* trouble. It produces an appreciable incidence of complications: immediate, such as hemorrhage,[23] infection and loss of skin;[24] and delayed, such as ulcerations and blocking of the urinary opening. More serious complications are fortunately rare, but they occur.

I cite only for its intrinsic interest, not to frighten expectant parents, the well-known case of a 7-month-old boy, one of a pair of identical twins, who was being circumcised by electrocautery. The current was too strong and burned the penis so badly that it was wholly ablated, flush with the abdomen. The desperate parents finally agreed to have this little boy transformed into a girl, through surgery and hormone treatment.[25] It is unlikely that so drastic a mischance will happen again soon.[26]

A particularly well-considered essay by the pediatrician E. Noel Preston concludes: "Routine circumcision of the newborn is an unnecessary procedure. It provides questionable benefits and is associated with a small but definite incidence of complications and hazards. . . . Circumcision of the newborn is a procedure that should no longer be considered routine."[27] Another physician, W.K.C. Morgan, ends an essay in the same journal with the words: "The teaching of the Koran and Bible, the mistaken beliefs of many in the medical profession, the intuition of woman (note: *woman*), and above all folklore, tradition and health insurance agencies support this ritual. Nevertheless let us remember that 98 times out of 100 there is no valid indication for this mutilation other than religion."[28]

It is only fair to note that each such statement in the medical literature inspires letters from other physicians, both of agreement and rebuttal. Having read both sides of the argument carefully, I come out convinced that there can be little wrong with keeping the foreskin that the habit of washing won't fix. Given a good chance that the genital area will be kept reasonably clean, regarding infant circumcision as a "health measure" is only to rationalize what is in fact a distressing mutilation of young infants.

But that is only one of a galaxy of such rationalizations. Specifically for those parents with whom this practice is not traditional and so must decide whether or not to circumcise, that decision involves motivations and repressions that rarely surface, perhaps for the very reason that they lie so deep and are potentially so painful. But that brings us back to where this essay began.

The Mothers

"I've never been able to forgive my mother for having that done to me," said my young friend in Tallahassee. His mother, not his father. Throughout the current medical discussion of circumcision, it's all mothers; not a word about fathers.

For it's the mothers who decide, in the hospital, talking with *their* obstetricians, *their* pediatricians, about *their* babies, still wholly their responsibility. Not the Jewish mothers. There no decision is needed, circumcision is taken for granted, and the fathers are ritually responsible. No Jewish boy would dream of blaming his mother for having him circumcised.

Why do gentile mothers have their infant sons circumcised?

It is strange that one of the commonest reasons they offer is that the penis *looks better* circumcised. I think that strange because those same mothers are horrified by all kinds of other mutilations practiced by peoples they regard as barbaric, for just such cosmetic reasons: tattooing, scarification, lengthening of the ear lobes—or in some African tribes, of the labia minora—lip plugs, and the like. Why do they condone *this* mutilation? Why do they think it cosmetic? In part for the curious reason that by now—for here in the U. S. we are in the second and third generation of white middle class circumcision—many mothers have never seen an uncircumcised penis. It would seem strange to them; they are afraid that their little boys would feel strange having one.

That is ironic, for Michaelangelo's David, the epitome of young male beauty, whose pictures adorn innumerable school textbooks—Michaelangelo's David is uncircumcised. Ostensibly Jewish, yet uncircumcisied. What was in Michelangelo's mind, making David so big, as big as Goliath; making him so old, no stripling as in the story; making him nude, a sinful state in Biblical times? Was that last just to show that he was uncircumcised?

Other changes are rung on this sentiment. A young woman about to have her first child said to me that if it was a boy she would want him circumcised "so that he would look like his father." And innumerable mothers have their sons circumcised so that they will look like other boys, so that they won't be embarrassed later when undressing with others "in the locker room."

But there are deeper issues, somewhat harder to come at since they involve in part what are still powerful social taboos.

Some years ago the English neurologist Henry Head and his co-workers showed that the glans of the penis lacks the receptors of fine sensory discrimination—light touch, small gradations of warmth or cold—what Head called the epicritic sensations. The glans conveys only protopathic sensations: of deep pressure, extreme heat or cold, and pain. That is, the glans responds only to coarse stimuli, yet with sensations that possess what Head spoke of as great "affective tone," whether exquisite pleasure or acute discomfort. The foreskin, however, like most other skin, has all the apparatus of fine as well as coarse sensory discrimination; and removing it takes away a considerable area of delicately responsive sensory surface.

The foreskin also provides a protective sheath for the glans, keeping its skin moist and tender. Its removal exposes the glans to the constant abrasion of clothing and keeps the skin dry. Hence the skin of the glans grows tough and coarse, with a further loss of sensitivity. Through both these effects, circumcision results in an appreciable loss of sensitivity and responsiveness of the male member.

There is also a mechanical consideration. "During the act of coitus the uncircumcised phallus penetrates smoothly and without friction, the prepuce gradually retracting as the organ advances."[29] An English physician likens penetration by the circumcised organ to thrusting the foot into a sock held open at the top, whereas for its intact counterpart it is like slipping the foot into a sock that has been rolled up.[30] Western women are horrified to hear of the practice in other parts of the world of female excision, clitoridectomy, particularly now that it is commonly believed that the clitoris is the main, if not the only source of female pleasure in coitus. Women are indignant that so much more damaging a practice should be taken to be in any way parallel to circumcision. We should realize however that what is done to males by circumcision involves a similar loss of sexual responsivity.

Having first encountered such considerations in the current medical literature, I was surprised to learn that this was familiar ground to the ancient rabbis. The great twelfth century Talmudists Judah Halevi, Judah of Barcelona and Moses Maimonides all agreed that the main object of circumcision was to encourage sexual restraint by lowering the sensitivity of the male organ and hence sexual pleasure. Thus Maimonides: "Circumcision simply counteracts excessive lust; for there is no doubt that circumcision weakens the power of sexual excitement, and sometimes lessens the natural enjoyment. . . . Our Sages (Bereshit Rabba, c. 80) say it distinctly: It is hard for a woman with whom an uncircumcised man had sexual intercourse to separate from him. This is, I believe, the best reason for the commandment concerning circumcision."[31]

One encounters also the contrary view, that circumcision involves a sexual advantage, directly for women, indirectly for men in making them more pleasing to women. The loss of sensitivity of the male organ can be viewed as a gain, since it increases staying power, the capacity to prolong the sex act. Also I have heard American woman express a preference for the circumcised organ on the grounds that it is neater, less messy and more available. Perhaps for these among other reasons, circumcision is reported to be spreading rapidly in parts of the Congo and Sudan in which it has only recently been introduced and has no ritual significance, because the women insist upon it in their sexual partners.

It is almost as though some women saw in the foreskin a competing vagina. And indeed Bryk reports an encounter just after an African circumcision rite, that almost says as much: "His girl comes . . . they talk all through the night. Early in the morning she gives him her hand and in parting says: 'I'll return tonight and then I'll give you my vagina. My dear man. Now I love you truly.'"[32]

Looking back over the last paragraphs I see emerging some degree of opposition between man-talk, whether by ancient rabbis or modern physicians, and woman-talk, much of it still unrecorded, and the little there is of it still largely filtered through males. It seems to bring out a male impression that circumcision decreases sexual pleasure in men that must compete with a female preference for this condition.

It is hard to estimate how much such considerations weigh when mothers decide to circumcise their infants. Whatever their reasons, the mothers do opt for this operation, at present almost universally in our country[33]—those who can afford it—yet sometimes with misgivings, knowing that they are handing over their babies for what may seem advisable, yet is surely unnecessary surgery.

What makes this decision easier is the widespread conviction that it doesn't hurt, that the infant feels no pain. The physician is likely to assure the mother that it doesn't hurt; and she thinks—and sometimes he thinks—that he knows. Here we encounter what I believe in fact to be a deeply planted and passionately defended rationalization.

As a biologist I have had to live with that kind of rationalization all my scientific life. We biologists in the course of experimenting sometimes have to decide whether to do things to animals that would hurt people if done to them. The question is whether those operations hurt the animals.

Biologists differ in their opinions about this. One must understand that there is no way whatever of finding out. There is no way—not even conceivably—of knowing what another animal feels. There is indeed no way to know what another person feels. The person can tell you; but then you only know what he has said, perhaps inaccurately, perhaps even to deceive you. There is no way at all to check up. A person's or animal's sensations are forever their own, forever locked within a private world of consciousness that science cannot penetrate—if indeed one concedes consciousness to another person or animal, since one can know only one's own. We can do no more than recognize what we take to be signals of pain in certain patterns of behavior—writhing, struggling, squirming, yelping, moaning, and in the case of human beings capable of it, speech—yet with no assurance that the pain is felt.

Many biologists, having done some violence to an animal and observed such behavioral signals, prefer to dismiss them as "reflex," particularly when dealing with a "lower" animal—in invertebrate such as a lobster, or a cold blooded vertebrate such as a frog or a fish. Such attitudes are not confined to biologists. Few of us hesitate to throw a live lobster into a pot of boiling water, in which it writhes and struggles for a while before dying; or to thread a worm on a hook, however much it writhes; or with that bait to hook a fish, and then work or cut out the barbed hook. Do those animals feel what we are doing to them, do they feel pain? There is no way of knowing; one assumes whatever one likes.

As for me, working in the laboratory, I decided long ago that if I did anything to an animal that would hurt if done to me, and the animal reacted much as I would react—except for speech—that I would rather assume that the animal felt pain than that it didn't. Hence I don't do such things to animals. Once, not knowing any better, I did something awful to a lobster—though perhaps not as awful as boiling one alive—and I shall never forget how it writhed as it died. So now the only operation I perform on an intact, unanesthetized animal is to kill it; and I take a lot of trouble to see that I kill instantly, with one stroke, an animal handled gently up to that moment.

So I wonder about those babies. Why does anyone think that circumcision doesn't hurt them? Well, they can't *say* it hurts, not yet having learned to speak; and they don't seem to remember it later. But then, few persons remember much that happened before they were two or three, and just about no one remembers anything before that; yet who has lived with babies and believes that they are not perfectly aware of all kinds of experiences that they won't remember later?

There is a third, supposedly scientific reason for thinking that very young infants do not feel pain. We are told that the nerve fibers of newborn infants have not yet acquired the myelin sheaths that will later insulate them from one another and help them to conduct excitation more rapidly. It has been suggested that for this reason newborn infants may not feel pain. But as it happens, the sensation of pain, unlike other skin senses, continues throughout life to be conducted, at least in part, by very fine nerve fibers that lack myelin sheaths. So none of these arguments makes much sense.[34]

I called an old friend, an obstetrician who long ago had taken care of my wife—and circumcised our son. "How do you do the operation?" I asked him. "Do you use a local anesthetic?" "Oh no!" he said. "Then doesn't it hurt?" I asked. "Well," he said, falling back upon a common physician-to-patient euphemism, "there's no doubt that the baby is uncomfortable." "Doesn't it squirm and struggle?" I asked. "Well," he said, "it can't!" -and then to my surprise, "As a matter of fact it's rather gruesome. We fasten the baby down in a form that holds him so that he can't move during the operation. But it takes only about 5 min." "Five minutes!" I said. "That sounds pretty long to me. I thought it only took a few seconds." "Would you like to see one done?" he asked. "Sure," I said. So we made a date.

A few mornings later I went to the hospital at the time he had mentioned. When I asked for him at the desk, I was told that he had been delayed. Would I wait over there? When I went over there, I found a young couple, the mother holding a lovely baby boy. "So you're waiting for Dr. ___, too!" she said with a bright smile; and with that I realized that her baby was going to be the patient.

We had about 20 min to wait, and chatted together, all happy and relaxed. The parents were very proud of their beautiful son. He had been a little premature, the mother explained, and so had to wait awhile before being circumcised. Now he was 6 weeks old and doing fine, as I could see. He had already more than doubled his birth weight.

Just then a middle-aged nurse came up to us, and asked for the baby. She began to walk off with it. The mother, still all smiles, started along with her; but the nurse stopped and said, kindly but firmly, "Please wait here." The mother looked distressed. "I thought I could go along!" she cried. "Oh, no!" said the nurse, "but we'll be back in about 10 min." And she went off with the baby.

That mother's face was a study. She sat down again, but now bolt upright, very tense, her face rigid, her eyes straight ahead. Her husband on the other hand was completely relaxed, even making a show of it, chuckling, patting her on her shoulder, telling her jovially that there was nothing to worry about. "Be a man!" he seemed to be saying to her; but she wasn't. She was a very worried woman. I wondered how many mothers had been through that before.

Then I was called, and found the doctor in a little surgery. The nurse was still holding the baby, who was quiet and relaxed; and I put on a sterile gown and cap. Then the infant was laid on a plastic form with a depression the shape of a child. His wrists and ankles were clipped into cuffs that held them gently but firmly. With that he began to cry bitterly; the restraint seemed to bother him at least as much as anything that happened later.

The surgeon laid a sterile sheet over the child's middle, with a circular opening exposing the genitals. As already said, the foreskin of such a young infant is usually too tight to be retractable, and often is attached to the glans. So after gently freeing the foreskin all around with a probe, the doctor slit it dorsally with scissors, so that it could be slipped back. There was a little bleeding. Then the shaft of the penis was inserted into the thimble-shaped end of a stainless steel rod, and the slit foreskin pulled forward over the outside of the rod. A circular steel ring was clamped tightly about the foreskin, crushing a narrow band of it between the clamp and the steel rod, just below where the foreskin would be severed. The physician explained that crushing the tissues in this way would cut the bleeding, help the cut ends of the foreskin to heal properly, and numb the nerves. The clamp was left on for 5 min. Then the foreskin was cut through all around, just above the clamp, and slipped off. The steel tool was removed, and the penis wrapped in a Vaseline-gauze dressing. The baby's wrists and ankles were freed, and the operation was over.

"Are you for it or against it?" the surgeon asked me afterward. "I suppose you're neutral." "I'm against it," I said. "So am I," he replied.

Yet nothing about this operation seemed to me horrifying. Clearly the infant was distressed, but seemingly as much by the restraint as anything else. There was no marked response to what I would have judged to be the most painful episodes—the moments of crushing and cutting of tissues. After the first bout of crying on being fastened down, it seemed to me more as though the infant were trying to withdraw into himself. To my astonishment, at one point right in the middle of the operation he seemed to be falling asleep![35]

Someone later showed me an interview with the psychiatrist Wilhelm Reich, in which he said: "Circumcision is one of the worst treatments of children. And what happens to them? They can't talk to you. They just cry. What they do is shrink. They contract, get away into the inside, away from that ugly world."[36]

And of course the operation is not the end of it. Barring complications, the circumcised infant is in for 3-5 days of soreness, his glans swollen, inflamed, and blue owing to the disturbed blood circulation.

And the mother? Under the usual conditions of a hospital delivery and circumcision, she hardly knows her baby yet. He is brought in to her periodically to nurse; but he is still in the hospital's care, not hers. She agrees to have him circumcised, or perhaps requests it, quite impersonally. It is a social decision, and remote. She doesn't know, and no one tells her, when or how it will be done. It all happens far off somewhere, between two feedings.

And yet. . . The infant comes back to her somewhat changed, fretful, withdrawn. One mother said to me, "His crying sounded different to me afterward." He has been hurt; a violence has been done to him. Many a mother wonders a little, worries a little, then puts it out of her thoughts. After all, everyone else does it, all her friends. And it was done for his sake, for his health, for his peace of mind later when he undresses in that locker room.

And yet. . . Suppose it was done for no good reason? Suppose all that was accomplished was the painful mutilation of a helpless infant? The very suggestion is an affront, an attack where the mother is most vulnerable. I shall probably not be forgiven this essay. And to have that attack come from sons! No wonder that those mothers in Tallahassee were upset with my young friend.

The Outcome

George Wald (November 18, 1906 – April 12, 1997) was an American scientist who is best known for his work with pigments in the retina. He won a share of the 1967 Nobel Prize in Physiology or Medicine with Haldan Keffer Hartline and Ragnar Granit.

I have come a long journey since Tallahassee. It is not yet over; I wonder whether it will ever be. There is a lot more to explore. Yet I should like to say where this encounter finds me now.

As I write this essay, we are working in the laboratory on the skins of frogs. We use just a small patch of skin in each experiment. The other day the thought occurred to me—I wouldn't dream of cutting that snippet of skin from a live frog. You couldn't bring me to do it. As it is, we kill the frog, take a piece of skin to work with, then come back later for another piece. It would probably make a better experiment to take a patch of skin from a live frog, and leave the rest on him until we wanted more.

But I couldn't do that. It would seem to me cruel. Yet frogs can't talk. Does it feel pain? Does it remember? I don't know; and there is no way that I can find out. It's just like those babies.

It's curious—and revealing—how few persons think about circumcision, or indeed about anything involving the genitalia, even their own. What do they even *look* like? Are they pretty much alike from person to person, or do they vary a little, or a lot? One hardly knows. At one point in writing this essay I looked through the shelves in our Biological Laboratory Library, through books on the senses, on neurophysiology and neuroanatomy, some of them medical textbooks. I was looking for what new information there might be on sensory responses from the glans penis and foreskin. Neither of those words was in the index of any of those books.

I hadn't thought at all about circumcision until that conversation in Tallahassee; but now that I have thought about it—it's just as with the frogs. I could not bring myself to have another infant of mine circumcised.

There is a complication, for I am a Jew, circumcised as is my son. A non-observing Jew, a non-believer in anything supernatural, yet deeply involved, a Bible reader—of both Testaments—and very much a Jew. For me there are special barriers against deciding not to circumcise; for it is hard to break with a tradition that one's ancestors have observed for thousands of years, however else one feels.

So I have looked into what it means for a Jew not to be circumcised. I find the position a little surprising.

For one thing, the son of a Jewish mother is wholly a Jew, regardless of circumcision. He can practice his Judaism in any form and to any extent he likes. He may take part in all observances, private and public. A Jewish father is obliged to have his son circumcised; but at thirteen that obligation passes to the son. Not to carry out that obligation is a *transgression*; the uncircumcised Jew is fully a Jew, but transgressing. Incidentally, Reform Judaism asks circumcision only for born Jews, not for those converted to Judaism.

A second discovery will surprise many Jews: the usual hospital circumcision does not fulfill the ritual requirement. It is only an operation, where what is needed is a consecration—a handing over by the Jewish father of his son, to enter the covenant of Abraham, sealed with the shedding of the son's blood. A hospital circumcision does nothing in this regard. A Jewish child who has already undergone such surgery would still need this ceremony and a token drawing of blood to fulfill the ritual requirement.

An added surprise for me has been to realize the relative relaxation with which great talmudists of past centuries viewed circumcision. I have already mentioned that Maimonides, the twelfth century codifier of the Talmud, considered the main point of circumcision to be, by weakening the organ of generation, to foster sexual restraint—an opinion with which other great rabbis of his time concurred.[37] Maimonides also allowed for the possibility that a grown boy might refuse to be circumcised, or a father for love of his son might neglect to have it done. After all, said Maimonides, it is "a very difficult operation." My rabbinic mentors agree that in former times and in other places Jews may have felt more relaxed about circumcision than do orthodox Jews now, perhaps goaded by the existence of reform Judaism, and the disastrous aftermath of a century of relaxed standards in central Europe.

It seems to me that a final consideration might bear upon this problem. Child sacrifice (to "Moloch") was a common rite among the ancient peoples of the Near East, and the Jews were forbidden it in the harshest terms (Leviticus 20:1-2). When God laid claim to all firstborn males, he specified that though those of the domestic animals were to be sacrificed, children were to be redeemed. As Moses, having been instructed by God, explained to the people: "I sacrifice to the Lord all the males that first open the womb; but all the first-born of my sons I redeem" (Exodus 13:15). And one of the ordinances that God gave to Moses along with the Ten Commandments states: "The firstborn of your sons you shall give to me. You shall do likewise with your oxen and your sheep: 7 days it shall be with its dam; on the eighth day you shall give it to me." (Exodus 22:29).

That command to sacrifice the male firstborn of a domestic animal, taking it from its mother when it is 5 days old, makes one wonder whether the prescribed circumcision of sons on the eighth day was once a form of redemption, the token sacrifice of the foreskin to substitute for sacrifice of the child. In any case the principle of redemption runs through these commandments, not only the obligatory redemption of sons, but "every firstling of an ass you shall redeem with a lamb . . ." (Exodus 13:13).

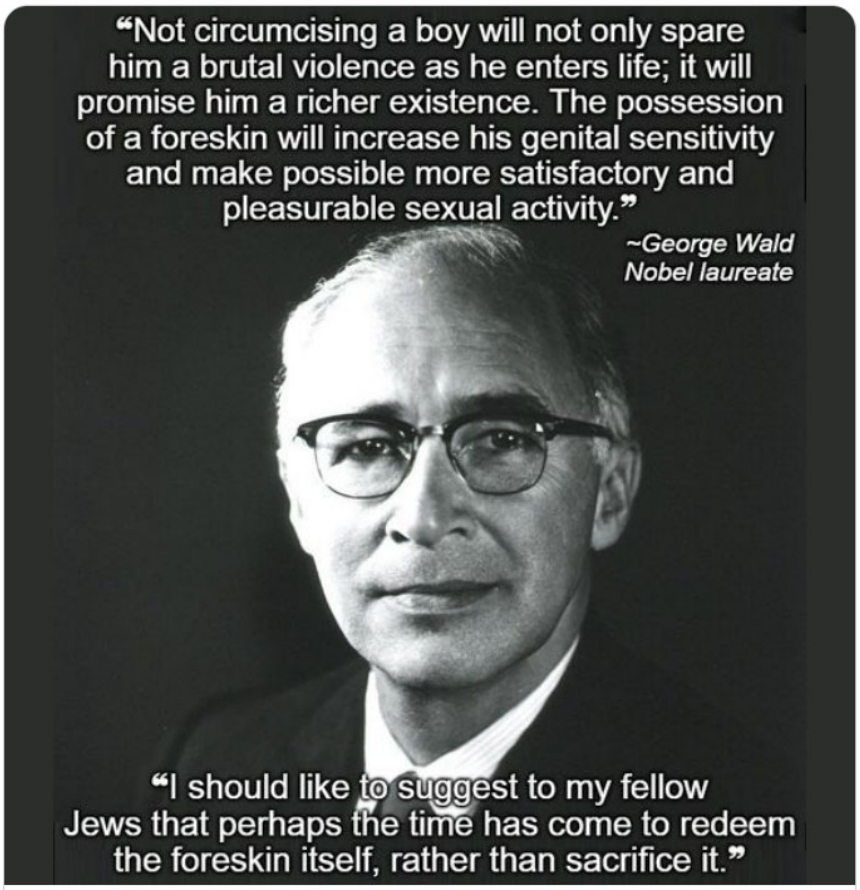
It is with the greatest hesitation, since I have no right and know so little, that I should like to suggest to my fellow Jews that perhaps the time has come to redeem the foreskin itself, rather than sacrifice it. Surely some substitute might be found for this rite, perhaps even involving a token drawing of blood from an older child, that would be preferable to this assault upon and mutilation of a newborn infant.

Since by now I would not circumcise even a Jewish infant, I would not dream of doing this to a gentile child. I would know no medical reason to deprive my sons of their foreskins, being confident that they would share with me habits of cleanliness that would make that unnecessary. If I had my children in grinding poverty and squalor, that might make a difference. I might then even *want* to have them circumcised, and perhaps would fail, for the usual reason that I could not pay. As with so many other things, those who might need circumcision are least likely to get it, and those who get it are least likely to need it. But even were I poor, if I had cleanly habits and some chance of passing them on to my sons, I would not circumcise them.

For it is a barbarous thing to meet a newly born infant with the knife, with a deliberate mutilation. And the part that is removed is not negligible; it has clear and valuable functions to perform. Not circumcising a boy will not only spare him a brutal violence as he enters life; it will promise him a richer existence. And that not only because the possession of a foreskin will increase his genital sensitivity and make possible more satisfactory and pleasurable sexual activity, but also because of the consideration with which this essay began: that the foreskin is the female element in the male.

To be sure, that is only a primitive insight, and has no standing in science. Yet that is hardly a criticism. What we consider to be male or female is largely cultural in any case; many of our conventional notions in this regard are now in flux and being challenged. This one has more basis in reality than most. Also unlike many unscientific interpretations of reality that are misleading and dehumanizing, this one can sustain, enrich and illuminate. It offers some redress where it is most needed, in a world increasingly devastated and threatened with destruction by a rampant machismo, a mindless exercise of organized aggressive maleness.

Every schoolchild knows that femaleness is determined genetically by the possession of two sex or X chromosomes (XX), and maleness by one together with a relatively empty Y chromosome (XY). Very rarely a male is born with an extra Y chromosome, so XYY. A few years ago, on somewhat questionable grounds, this condition was reported to be correlated with violent behavior.



Recently a research project was set up at the Harvard Medical School, to type the chromosomes of a large number of infants and so find a group, which is XYY. The idea was then to tell the parents and follow the behavior of the children, to see whether any special tendencies toward violence emerged. Some other research workers at Harvard Medical School objected strongly to this project, feeling that the study itself might prejudice the children's behavior and relationship with their parents. A bitter controversy followed, that ended with the director of the project terminating it.

While this dispute was at its height, Dr. Michael Mage of the National Cancer Institute wrote a letter to Science magazine to say that all that concern with the XYY syndrome provides a fine example of the way research workers in medicine pick strangely peripheral, esoteric topics to study. Our real concern, said Dr. Mage, should be with the XY *syndrome*, which afflicts half of humanity including himself, and is known to be strongly correlated with war and other forms of criminal violence.

Just so. Maleness is our problem, excessive maleness. The circumcised organ is only the beginning of it, and kept hidden. What are displayed, like so much male plumage, are the penis surrogates and aggrandizements: the guns; the cars, named for predatory beasts, driven to and from work as though they were PT boats; the flaunting of power and status; the devastation of the earth and the cultivation of a technology of death and destruction beyond any former imagining, all in the pursuit of an obsessive accumulation of wealth far beyond any possibility of use—all the brutal, gaudy, pretentious and infinitely dangerous panoply of male aggression that now envelops and threatens our lives.

This is no time to circumcise males. They need all the female element they can get.

For every child is born into the world with much of one sex and a little of the other. The mistake is by a mutilation to take that little of the other sex away. It should be left as nature evolved it, as in the child, so that all our lives we can go on being much of one sex, and always a little of the other.

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[1]. Ed. Note: This was Van Lewis, who had been a student of Wald's at Harvard.
[2]. Griaule M. (1965) Conversations with Ogotemmeli. Oxford University Press, Oxford, pp 22–23.
[3]. Breasted JH. (1946) The Dawn of Conscience. New York, NY: Scribner, p 353.
[4]. Ed. Note: An American textbook on obstetrics documents a case where the obstetrician circumcises the breech infant while waiting for the head to be born: Schaffer AJ, Avery MJ. (1977) Diseases of the Newborn, 4th ed. Philadelphia, PA: Saunders, p 420.
[5]. Note: Urine provides a sterile medium for washing under the foreskin that does not demand retracting it. On the contrary it can be pulled forward and held closed so that the child urinates into it, ballooning it, until released.
[6]. Guide for the Perplexed, M. Friedlander trans. (1904) London: Routledge and Kegan Paul, p 378.
[7]. Freud S. (1938) Totem and Taboo. New York, NY: Random House.
[8]. Symbolic Wounds. (1954) Puberty Rites and the Envious Male. Glencoe, IL: Free Press, p 112.
[9]. Guide for the Perplexed M. (1904) Friedlander Translation. London: Routledge and Kegan Paul, pp 378–379.
[10]. Moses and Monotheism. (1939) New York, NY: Alfred A Knopf, p 215.
[11]. Beyond God the Father. (1973) Boston, MA: Beacon Press, p 195.
[12]. Guide for the Perplexed M. (1904) Friedlander Translation, 2nd ed. London: Routledge and Kegan Paul, p 216.
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[21]. Oster J. (1968) Further fate of the foreskin. Arch Dis Childhood. 43:200.
[22]. Oster, ibid.
[23]. A 9 pound infant has only 12 oz. of blood. Loss of 20% (only 2.4 oz.) can lead to shock, heart failure and death.
[24]. Ed. Note: Loss of skin puts the complication rate at 100%.
[25]. Money J, Ehrhardt AA. (1972) Man and Woman, Boy and Girl. Baltimore, MD: Johns Hopkins University Press, pp 18–123
[26]. Ed. Note: An article in 1989 documents four cases. (Gearhart JR, Rock JA. (1989) Total ablation of the penis after circumcision with electrocautery: A method of management and long-term followup. J Urol. 142:799–801).
[27]. Preston EN. (1970) J Am Med Assn. 213:1858.
[28]. Morgan WKC. (1965) J Am Med Assn. 193:224.
[29]. Morgan WKC. (1965) J Am Med Assn. 193:223–224.
[30]. Whiddon D. (1953) Lancet. 2:337.
[31]. Guide for the Perplexed M. (1904) Friedlander Translation. London: Routledge and Kegan Paul, p 378.
[32]. Bryk F. (1928) Neger-Eros. Berlin: Marcus and Weber, p 59; cited in Bettelheim B: Symbolic Wounds, p 163.
[33]. Ed. Note: Thirty-five years later, the rate is down close to 50%, and in some parts of the country, much lower.
[34]. Ed. Note: Wald was ahead of his time. In 1987, Anand and Hickey wrote an article in the N Engl J Med (317:1321–1329) that documented cortisol levels with circumcision as high or higher than those in adults with great pain, and now it is generally recognized that infants suffer under the knife.
[35]. Ed. Note: The crushing has now been documented as excruciating pain, and the infant has just gone into a coma.
[36]. Reich W. (1967) Reich Speaks of Freud. New York, NY: Farrar, Straus and Giroux, p 29.
[37]. cf. Philo of Alexandria: The same view, a millennium earlier.

About Network for Church Monitoring

Network for Church Monitoring is a progressive non-profit organization that seeks to bring together individuals from across the globe – no matter whether theistic, atheistic, or agnostic – who are deeply concerned with the Vatican and the Christian Right's tireless work to insinuate their values into every aspect of public policy debate at every level of government.